EdPsych Connect

Understanding Emotions and Behaviours

A Comprehensive Guide for Parents and Schools

Supporting Children and Young People with
Emotionally Based School Non-Attendance (EBSNA)

A framework for understanding the biological basis of emotions,

affect scripts, shame responses, and evidence-based interventions

Dr Scott I-Patrick
DEdPsych CPsychol
Chartered Child and Adolescent Educational Psychologist

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Contents

Section 1: Introduction and Context for EBSNA

Section 2: The Biological Basis of Emotions

Section 3: Understanding Affect Scripts

Section 4: Tomkins' Theory of Affect Script

Section 5: The Central Blueprint Rules

Section 6: Nathanson's Compass of Shame

Section 7: The Polyvagal Perspective

Section 8: The Morning Routine and Affective Cascade

Section 9: Masking and Delayed Presentation

Section 10: The Role of Transitions

Section 11: The Social Discipline Window

Section 12: Parental and Staff Affect Scripts

Section 13: A Functional Analysis Framework

Section 14: Recovery and Reintegration

Section 15: Practical Scenarios and Strategies

Section 16: Quick-Reference Guide

Section 17: Resources and References

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 1: Introduction and Context for Emotionally Based School Non-Attendance

Children and young people with, or requiring, an Education, Health, and Care Plan (EHCP) often navigate complex emotional and behavioural landscapes shaped by their unique biographies. Their lived experiences, marked by academic struggles, social challenges, and sensory sensitivities, profoundly influence how they perceive school and interact with their environment. Emotions are not just fleeting feelings; they are biological responses hardwired into our autonomic nervous system, triggered automatically by what we perceive in our environment or by memories of past experiences. For these children, school can often feel like a battleground of overwhelming emotions, where each day presents new triggers for anxiety, frustration, and shame.

What is Emotionally Based School Non-Attendance?

Emotionally Based School Non-Attendance (EBSNA) is a term used to describe a pattern of school attendance difficulties that arise from emotional distress rather than deliberate truancy or parental withdrawal. It is important to understand that EBSNA is not a choice; it is an involuntary response to perceived threat or overwhelming distress. The child's nervous system has learned, through repeated experiences, that school represents danger, discomfort, or emotional pain.

> *Key principle: EBSNA is not about a child choosing not to attend school. It is about a child whose emotional and physiological responses make attendance feel impossible.*

This handout aims to provide parents and teachers with the tools to look beyond presenting behaviours, recognising how past experiences shape emotional responses and drive behaviour. By exploring key psychological theories, we aim to equip you with insights that will help you understand and support these children with empathy and targeted interventions. The quality of your relationship with the child plays a crucial role in helping them navigate their emotional world, fostering trust and creating opportunities for positive engagement.

Why This Framework Matters

Understanding the theoretical underpinnings of emotional responses provides several advantages. Firstly, it helps shift the narrative from blame to understanding. When we recognise that behaviours are driven by automatic biological processes rather than deliberate defiance, we can respond with empathy rather than frustration. Secondly, it provides a common language for parents, schools, and professionals to discuss what might be happening for a child. Finally, it offers clear intervention points where adults can disrupt negative cycles and foster more positive emotional experiences.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 2: The Biological Basis of Emotions

Emotions are not simply feelings that we choose to have. They are powerful, automatic responses that guide how we respond to the world around us. The term 'affects' refers to biological markers hardwired into our autonomic nervous system, meaning they are automatic responses that occur without conscious choice. When a child perceives something in their environment, whether it is a task at school, a social interaction, or a sensory experience, these affects are triggered instantly.

By the time we experience an emotion positively or negatively, it has already been set in motion by the immediate stimulus or by thoughts and memories from previous experiences that are stored in the brain. These responses are not choices; they happen automatically, influencing how a child reacts to their surroundings.

> *Critical insight: By the time a child displays a behavioural response, the emotional trigger has already occurred. The behaviour is the visible tip of an invisible emotional iceberg.*

The Autonomic Nervous System

The autonomic nervous system (ANS) operates largely outside our conscious control, regulating essential bodily functions including heart rate, breathing, digestion, and crucially, our threat responses. The ANS has two main branches: the sympathetic nervous system, which activates our fight or flight response, and the parasympathetic nervous system, which promotes rest and recovery.

When a child perceives threat, whether real or remembered, the sympathetic nervous system activates rapidly. Heart rate increases, breathing becomes shallow, muscles tense, and the body prepares for action. This physiological state is incompatible with calm, focused learning. Understanding this helps explain why a child in emotional distress cannot simply 'choose' to engage with a lesson or 'decide' to come into school.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 3: Understanding Affect Scripts

While affects are the immediate, biological responses to stimuli, scripts are the patterns of emotional responses that develop over time based on repeated experiences. Scripts are essentially the emotional 'blueprints' that guide behaviour, formed from the child's past encounters with similar situations.

For example, a child who repeatedly experiences failure in a particular subject may develop a script that associates that subject with distress and avoidance. These scripts can include deeply embedded thoughts like 'I am stupid' or 'I always fail,' which shape how the child views themselves and the tasks they encounter.

Scripts as Behavioural Guides

Scripts act like a guidebook that directs a child's actions and reactions. Positive scripts, shaped by experiences of joy, interest, and success, guide children towards tasks and interactions that they find engaging and rewarding. Conversely, negative scripts, formed from experiences of shame, fear, or frustration, steer children away from challenging tasks or situations, fostering avoidance, withdrawal, and defensive behaviours.

The interplay of these scripts significantly impacts a child's self-concept, motivation, and overall engagement in learning.

Affective Resonance and the Loop of Emotions

Affective resonance means that once one script is triggered, it can activate a cascade of related emotions, reinforcing negative beliefs and behaviours. For instance, a script triggered by a challenging task might bring up feelings of fear, followed by thoughts of inadequacy like 'I cannot do this,' which then perpetuates the cycle of avoidance.

This loop can be difficult to break, especially if the child feels misunderstood or unsupported by the adults around them. Without understanding the underlying affects and scripts, supporting adults may miss critical opportunities to disrupt these negative cycles.

> *Key insight: Negative scripts create self-fulfilling prophecies. A child who believes they will fail is more likely to avoid, withdraw, or give up, which then confirms their belief that they cannot succeed.*

The Role of Adults in Breaking the Negative Loop

To break the loop of negative affective resonance, it is crucial that supporting adults understand the child's emotional world. Disrupting negative scripts requires knowing enough about the child to engage them in ways that trigger positive affects, such as interest and excitement. This is where the quality of relationships plays a crucial role; supportive relationships built on trust, empathy, and understanding provide the foundation for positive interventions that can shift a child's emotional and behavioural scripts towards more adaptive and positive patterns.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

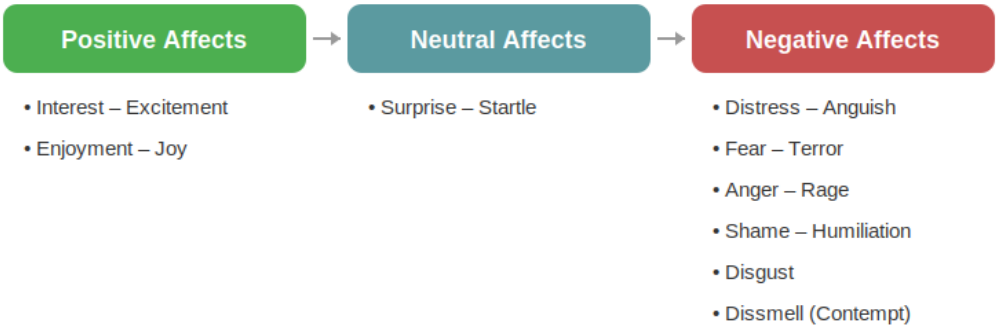
Section 4: Tomkins' Theory of Affect Script

Following the foundational understanding of affects as automatic biological responses and scripts as learned guides of behaviour, Tomkins' Theory of Affect Script provides a deeper categorisation of these emotional responses. Tomkins identified nine affect scripts, categorised into three groups: positive, neutral, and negative. These scripts help explain how repeated emotional experiences influence behaviour, guiding children either towards engagement and connection or towards avoidance and withdrawal.

The Nine Affect Scripts

Positive Affects	Neutral Affects	Negative Affects
Interest – Excitement Enjoyment – Joy	Surprise – Startle	Distress – Anguish Fear – Terror Anger – Rage Shame – Humiliation Disgust Dissmell (Contempt)

Figure 1: Mapping Emotional Responses – The Nine Affect Scripts



How Affect Scripts Manifest in Children's Behaviour

Children are naturally drawn to experiences that trigger positive affects, such as joy and excitement. These affects not only motivate engagement but also foster a sense of competence, connection, and a positive self-concept. However, persistent academic or social struggles can steer children towards negative scripts. For instance, a child who repeatedly encounters failure, social rejection, or overwhelming sensory experiences may develop scripts dominated by distress, shame, or fear. Over time, these negative scripts can lead children to anticipate failure and rejection, fostering hypervigilance, avoidance, and a deep sense of helplessness.

Section 5: The Central Blueprint Rules of Tomkins' Affect Theory

Tomkins' Central Blueprint outlines four fundamental rules that guide emotional regulation, emphasising the importance of managing automatic emotional responses to shape scripts positively. These rules are essential for understanding how to support children's emotional well-being, particularly for those who have experienced repeated negative triggers.

The Four Rules of the Central Blueprint

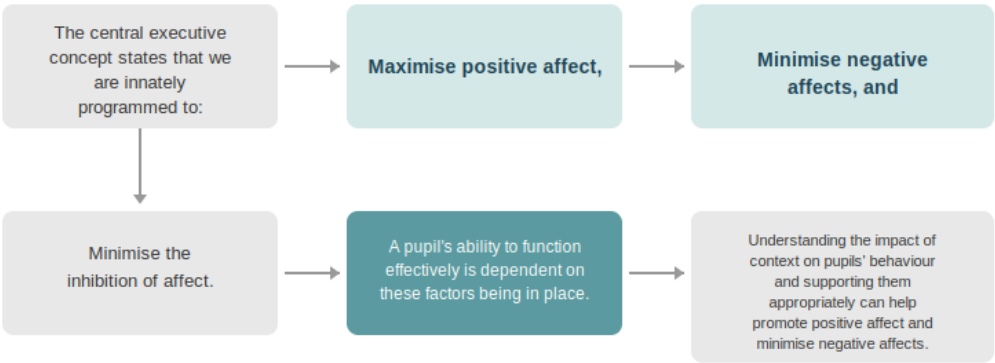


Figure 2: The Central Blueprint of Emotional Regulation

1. **Maximise Positive Affect:** Create environments and interactions that trigger positive affects, such as joy and excitement. Engaging the child's interests, strengths, and preferences is crucial in promoting positive scripts.
2. **Minimise Negative Affect:** Reduce triggers of negative affects such as fear, shame, or distress. Identify and modify environmental factors that contribute to negative emotional responses.
3. **Minimise Inhibition of Affect:** Avoid forcing children to mask or hide their true feelings. Suppression can lead to a build-up of unresolved emotions that may later manifest as behavioural outbursts or internalised distress.
4. **Maximise Expression of Affect:** Create safe spaces where children feel free to express their emotions without fear of judgement or punishment. Open expression helps children better understand their feelings.

Practical application: Every interaction with a child is an opportunity to either reinforce negative scripts or help build positive ones. By consciously applying these four rules, adults can systematically improve a child's emotional landscape.

Section 6: Nathanson's Compass of Shame

Nathanson's Compass of Shame expands on Tomkins' Affect Theory by specifically addressing how individuals respond to feelings of shame, a particularly powerful and interpersonal negative affect. Shame is an automatic emotional response that often arises from past experiences of failure, rejection, or unmet expectations. For children, especially those with additional needs, shame can be a frequent and deeply impactful emotional trigger, shaping how they interact with their environment and perceive themselves.

The Four Shame Responses

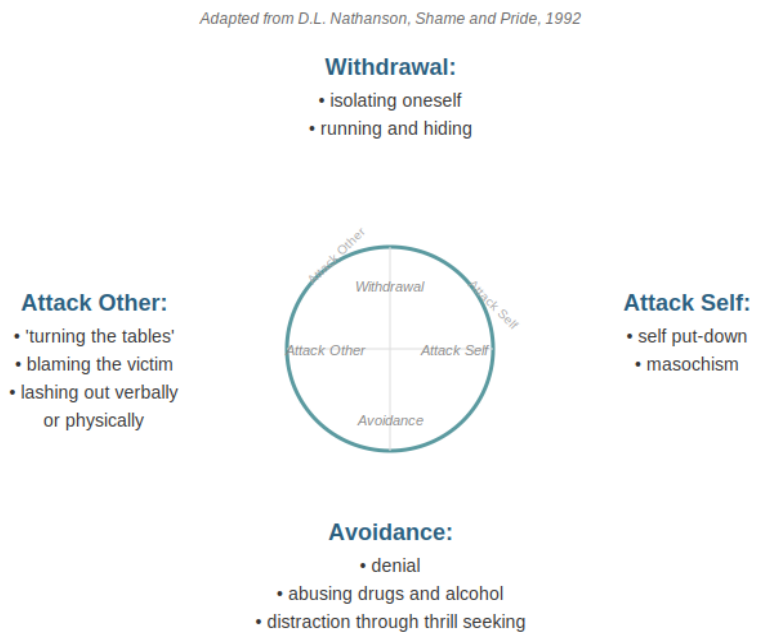


Figure 3: Nathanson's Compass of Shame – The Four Responses to Shame

Nathanson identified four common responses to shame, each representing a script that children may adopt as a way of managing overwhelming feelings of unworthiness:

1. **Withdrawal:** The child retreats into themselves, avoiding tasks, social interactions, or any situation that might trigger further shame. This often manifests as school avoidance, isolation, or emotional withdrawal.
2. **Avoidance:** The child engages in distractions or behaviours that divert attention away from their perceived flaws. This can include excessive daydreaming, avoidance of responsibilities, or over-reliance on screens.
3. **Attack Self:** The child turns shame inward, blaming and criticising themselves. This includes self-deprecating thoughts like 'I am stupid' or 'I cannot do anything right,' severely impacting self-esteem.

4. **Attack Others:** The child directs shame outward, becoming aggressive or blaming others for their discomfort. This defensive response serves as a shield to protect a fragile sense of self.

Critical understanding: These shame responses are not conscious choices but automatic reactions shaped by past negative experiences. Recognising this helps adults understand that these behaviours are deeply rooted in emotional experience and are not simply acts of defiance.

The Compass of Shame Diagram

The Compass of Shame can be visualised as four directional responses emanating from a central experience of shame. Withdrawal and Attack Self represent internalising responses, while Avoidance and Attack Others represent externalising responses. Understanding where a child tends to fall on this compass helps adults tailor their interventions appropriately.

Section 7: The Polyvagal Perspective

While Tomkins and Nathanson provide frameworks for understanding emotional scripts and shame responses, Polyvagal Theory, developed by Dr Stephen Porges, offers a crucial physiological perspective that helps explain why children experiencing EBSNA find it so difficult to 'just go to school.'

Understanding the Autonomic Nervous System Hierarchy

Polyvagal Theory describes a hierarchy of autonomic states that evolved to help us respond to our environment:

1. **Ventral Vagal (Social Engagement):** When we feel safe, this system is active. We can engage socially, think clearly, learn effectively, and regulate our emotions. This is the state required for successful school attendance.
2. **Sympathetic (Fight or Flight):** When we perceive threat, this system activates. Heart rate increases, muscles tense, and we prepare to fight or flee. Learning becomes extremely difficult in this state.
3. **Dorsal Vagal (Shutdown/Freeze):** When threat feels overwhelming and inescapable, this ancient system activates. The child may appear withdrawn, disconnected, or 'zoned out.' This is often mistaken for defiance or laziness.

Neuroception: Detecting Safety and Threat

A key concept in Polyvagal Theory is 'neuroception,' the unconscious process by which our nervous system evaluates risk in our environment. This evaluation happens below conscious awareness, which is why children often cannot explain why school feels threatening. Their nervous system has learned, through repeated experiences, to detect threat in the school environment, even when adults perceive it as safe.

> *Important insight: A child's neuroception may detect threat in situations that adults consider perfectly safe. This is not imagination or manipulation; it is the nervous system responding to learned associations. The child's body genuinely perceives danger.*

Implications for Supporting Children with EBSNA

Understanding Polyvagal Theory has profound implications for supporting children. Firstly, we cannot reason or reward a child out of a threat response. When the sympathetic or dorsal vagal systems are activated, the parts of the brain responsible for logical thinking are offline. Secondly, safety must be felt, not just promised. Telling a child that school is safe does not change their neuroception. They need repeated experiences of actual safety. Finally, co-regulation precedes self-regulation. Children learn to regulate their nervous systems through relationships with calm, regulated adults. Before a child can manage their own state, they need adults who can provide a calming, attuned presence.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 8: The Morning Routine and Affective Cascade

For many families experiencing EBSNA, mornings become battlegrounds. Understanding the affective cascade that occurs during morning routines can help parents and schools intervene more effectively.

The Cascade in Action

Consider what happens for a child with established negative school scripts as morning unfolds:

1. The alarm goes off, immediately triggering anticipatory anxiety about the day ahead.
2. This anxiety activates memories of previous difficult school days through affective resonance.
3. Shame scripts activate as the child feels inadequate for struggling with something peers seem to manage.
4. The sympathetic nervous system activates, causing physical symptoms such as stomach ache, headache, or nausea.
5. The behavioural response emerges: withdrawal, conflict with parents, or physical refusal to move.
6. Parents become stressed and frustrated, which the child's nervous system detects, escalating the threat response.
7. The cycle reinforces the child's script that mornings and school are dangerous and overwhelming.

The physical symptoms are real. When the autonomic nervous system activates in response to perceived threat, children genuinely experience pain, nausea, and discomfort. These are not invented or exaggerated; they are the body's natural response to threat.

Breaking the Morning Cascade

Intervening in the morning cascade requires understanding that once the cascade has begun, it is extremely difficult to reverse. Prevention is more effective than intervention in the moment. Strategies include preparing for the next day the evening before when anxiety is lower, reducing morning demands and decisions, maintaining calm and regulated parental presence, avoiding reasoning or negotiating when the child is already dysregulated, and considering whether the transition to school can be modified to reduce threat cues.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 9: Masking and Delayed Presentation

Many children, particularly autistic children and girls, are able to mask their distress throughout the school day, presenting as coping or even thriving in the school environment. However, the effort required for this masking is exhausting and unsustainable, often leading to significant distress at home.

Understanding Masking

Masking refers to the conscious or unconscious suppression of natural responses in order to appear 'normal' or to meet social expectations. For children in school, this might include suppressing stimming behaviours, forcing eye contact despite discomfort, hiding confusion or distress to avoid standing out, copying peers' social behaviours without genuine understanding, and using all available cognitive resources to appear regulated.

This links directly to Tomkins' third rule about minimising the inhibition of affect. When children suppress their genuine emotional responses all day, there are significant consequences.

The After-School Explosion

Parents often describe children who appear fine according to school reports but then 'explode' when they get home. This is sometimes called 'after-school restraint collapse.' The child has used all their regulatory capacity to hold it together during the day, and once they reach the safety of home, the suppressed emotions are released.

> *Why school says 'everything is fine': School staff see a regulated child because the child is expending enormous effort to appear that way. This does not mean the child is coping; it means they are masking. The true picture of the child's emotional state is often only visible at home.*

Implications for Adults

When schools report that a child seems fine, this should not be taken as evidence that the child is coping. Instead, adults should look for subtle signs of strain during the school day, take parental reports of home difficulties seriously, understand that masking is not sustainable and often precedes complete breakdown, and recognise that the child's capacity for masking may diminish over time, leading to school refusal.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 10: The Role of Transitions

Transitions are often significant triggers for EBSNA. Understanding why transitions are particularly challenging helps adults provide appropriate support.

Types of Challenging Transitions

Daily transitions include moving from home to school (the most significant for EBSNA), between lessons or activities, from break time back to classroom, and between different physical spaces within school.

Larger transitions include the return to school after weekends or holidays, moving between year groups, changing schools (primary to secondary being particularly challenging), and changes in staffing or classroom arrangements.

Why Transitions Trigger Distress

Transitions involve uncertainty, and uncertainty activates threat responses. For a child with negative school scripts, each transition represents a moment when threat might increase. The child cannot predict what will happen next, who they will encounter, or whether they will cope with what is coming.

Additionally, transitions often involve leaving a regulated state. A child who has finally settled in one lesson must now disrupt that state to move to another context. For children who find regulation difficult, this repeated disruption is exhausting.

> *The September phenomenon: Many children manage the summer term reasonably well but struggle significantly after the summer holidays. The long break allows anxiety to build, and the transition back involves multiple changes: new classroom, new teacher, new peers in class, new expectations. Each of these changes amplifies uncertainty and threat.*

Supporting Transitions

Effective transition support involves predictability through visual timetables, advance notice of changes, and consistent routines. It also requires reducing the number of transitions where possible, allowing longer settling time after transitions, using transitional objects or practices that provide continuity, and preparing for larger transitions well in advance with visits, social stories, and graduated exposure.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 11: The Social Discipline Window

The Social Discipline Window, developed within the restorative practices framework, provides a valuable lens for understanding how adults' responses to children can either support or undermine their emotional development and sense of agency. This model is particularly relevant when supporting children experiencing EBSNA, as it emphasises the importance of working with children rather than doing things to them or for them.

Understanding the Four Quadrants

The Social Discipline Window describes four approaches based on two dimensions: the level of control (limit-setting, discipline) and the level of support (encouragement, nurture) that adults provide.

TO **Punitive** High Control + Low Support *Stigmatising, disrespectful*	**WITH** **Restorative** High Control + High Support *Respectful, collaborative*
NOT **Neglectful** Low Control + Low Support *Indifferent, passive*	**FOR** **Permissive** Low Control + High Support *Rescuing, over-protective*

Figure 2: The Social Discipline Window – Four Approaches to Working with Children

The Four Approaches Explained

1. **Punitive (TO):** Adults do things to children. High expectations without adequate support. Characterised by punishment, blame, and consequences without relationship repair. This approach triggers shame responses and reinforces negative scripts.
2. **Permissive (FOR):** Adults do things for children. High support without appropriate expectations. While well-intentioned, this approach can undermine children's sense of agency and competence. It does not help children develop the skills to manage challenges.
3. **Neglectful (NOT):** Adults do not engage with children. Low expectations and low support. Children are left to struggle without guidance or nurture. This approach communicates that the child does not matter.
4. **Restorative (WITH):** Adults work with children. High support combined with high expectations. Children are involved in problem-solving and decision-making. This approach builds autonomy, competence, and connection.

The WITH approach is optimal because it respects the child's agency whilst providing the support and structure they need. Children are included in discussions about what is happening and what might help. Their voice matters.

Applying the Social Discipline Window to EBSNA

When supporting a child experiencing EBSNA, the temptation can be to slip into TO (forcing attendance through consequences) or FOR (removing all expectations and allowing complete avoidance). Neither approach addresses the underlying emotional needs.

A WITH approach involves having honest conversations with the child about what is difficult, collaboratively identifying what might help, maintaining expectations for engagement whilst adjusting how engagement looks, involving the child in planning their return or modified attendance, and recognising the child as an expert in their own experience.

This approach aligns with the Central Blueprint rules by maximising expression of affect (the child's voice is heard) whilst working towards positive outcomes.

Section 12: Parental and Staff Affect Scripts

Adults bring their own affect scripts to interactions with children. Understanding this dynamic is essential for effective support.

Parental Affect Scripts

Parents' own experiences of school shape how they respond to their child's difficulties. A parent who experienced school positively may struggle to understand why their child finds it so difficult. They may inadvertently minimise their child's distress because it does not match their own experience. Conversely, a parent who also found school distressing may find their own negative scripts activated. This can lead to either over-identification with the child's distress or avoidance of addressing the issue because it triggers their own difficult memories.

Additionally, parents often experience shame when their child is struggling with school attendance. This shame can manifest through the Compass of Shame responses: withdrawal from school communication, avoidance of the issue, self-blame and inadequacy feelings, or anger directed at school staff.

> *For parents: Your emotional responses to your child's school difficulties are valid and understandable. Recognising your own scripts helps you respond more effectively to your child's needs rather than reacting from your own emotional history.*

Staff Affect Scripts

School staff also carry their own affect scripts, which influence how they respond to children experiencing EBSNA. A teacher may experience shame when a child in their class refuses to attend, interpreting it as rejection of their teaching. Staff members facing pressure around attendance data may feel fear and anxiety, which can lead to punitive responses. Previous difficult experiences with families can create scripts that influence how staff interpret current situations.

When staff recognise their own emotional responses, they can separate their feelings from the child's needs and respond more therapeutically.

Creating Space for Adult Reflection

Both parents and staff benefit from reflective spaces where they can examine their own responses without judgement. Questions that support this reflection include: What does this situation trigger in me? What are my beliefs about school attendance and where do they come from? What shame responses do I notice in myself? How might my emotional state be affecting the child?

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 13: A Functional Analysis Framework

Building on the theoretical foundations explored in this handout, this section provides a structured framework for analysing an individual child's presentation. This framework can be used by parents, school staff, and professionals to develop a shared understanding of what might be happening for a specific child.

The Analysis Questions

Domain	Questions to Consider
Observable Behaviours	What specific behaviours are we seeing? When and where do they occur? What triggers seem to precede them?
Underlying Affects	Which of the nine affects might be driving this behaviour? Is this primarily fear, shame, distress, or a combination? What autonomic state is the child likely in?
Scripts	What scripts might have formed from past experiences? What beliefs about self, school, or others might be operating? What internal dialogue might the child be experiencing?
Shame Responses	Where does the child tend to fall on the Compass of Shame? Is the response primarily internalising or externalising? Has this pattern changed over time?
Environmental Triggers	What aspects of the environment trigger negative affects? Are there sensory factors, social dynamics, or academic demands? What times, places, or people are associated with difficulty?
Protective Factors	What triggers positive affects for this child? Which relationships feel safe? What contexts help the child feel regulated?
Intervention Points	Where in the cascade can we intervene? What would maximise positive affect? What would minimise negative triggers?

Figure 3: Functional Analysis Framework for Understanding EBSNA

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 14: Recovery and Reintegration

Supporting a child back into school attendance requires careful planning that takes account of the emotional and physiological processes outlined in this handout. Rushed or poorly planned reintegration often fails and can worsen the child's negative scripts.

Principles of Effective Reintegration

1. **Safety First:** The child's nervous system needs to experience school as safe before regular attendance can resume. This requires addressing the environmental factors that trigger threat responses.
2. **Gradual Exposure:** Successful reintegration typically involves graduated steps, starting with what the child can manage and building slowly. Pushing too fast reactivates threat responses and reinforces negative scripts.
3. **Positive Affect Focus:** Reintegration should prioritise creating positive experiences at school. The goal is to build new, positive scripts that can compete with the established negative ones.
4. **Relationship Priority:** Identifying and strengthening connections with trusted adults at school provides the co-regulation support the child needs.
5. **Flexibility:** Plans need to be responsive to the child's state. On difficult days, maintaining some connection with school, even if modified, is more valuable than pushing for full attendance.
6. **Child Voice:** Working WITH the child, as described in the Social Discipline Window, ensures that their expertise about their own experience informs the plan.

What Reintegration Might Look Like

Depending on the severity and duration of the EBSNA, reintegration might begin with simple exposure without pressure, such as walking past the school or visiting during quiet times. It may progress to brief visits to a safe space within school with a trusted adult, then to attending preferred activities or lessons for limited periods. Gradually, attendance can extend to longer periods and eventually include more challenging contexts.

> *Important: The pace of reintegration should be determined by the child's capacity, not by external timetables. A child who is pushed too fast will typically regress, and recovery takes longer than it would have with a more gradual approach.*

Creating New Positive Scripts

The ultimate goal of reintegration is not just physical presence at school but the development of new positive scripts that associate school with safety, competence, and connection. This requires repeated positive experiences that build new neural pathways and emotional associations. Every positive school experience contributes to this process, whilst every negative experience risks reinforcing the old scripts.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 15: Practical Scenarios and Strategies

This section provides practical scenarios that demonstrate how understanding affects and scripts can inform effective strategies to support children's emotional and behavioural needs.

Scenario 1: Classroom Task Avoidance

Situation: A student frequently avoids participating in reading aloud during class, displaying signs of anxiety and avoidance behaviours.

- **Recognise the Affect and Script:** The child's avoidance is an automatic response triggered by past negative experiences, not a deliberate refusal to engage.
- **Supportive Intervention:** Offer alternative ways to participate, such as reading in pairs or pre-reading at home, to reduce the stressor and minimise negative affect.
- **Reinforce Positive Affect:** Celebrate small successes to trigger positive affects of joy and achievement, helping to rewrite the negative script.

Scenario 2: Morning Refusal

Situation: A child refuses to get out of bed on school mornings, complaining of stomach ache and becoming distressed when parents try to encourage them.

- **Recognise the Physiological Response:** The stomach ache is real. The autonomic nervous system has activated in anticipation of threat, causing genuine physical symptoms.
- **Avoid Escalation:** Parental stress will be detected by the child's nervous system and increase their threat response. Stay calm and regulated.
- **Work WITH:** Rather than forcing (TO) or giving up (FOR), explore with the child what might help. What feels most difficult? What small step might be possible?

Scenario 3: Aggressive Behaviour at Home

Situation: A child reacts with anger and aggression during homework time, expressing frustration with tasks they find difficult.

- **Identify the Shame Response:** Recognise that aggressive behaviour is a manifestation of the 'attack others' shame response, where anger is directed outward as defence against feelings of failure.
- **Intervention through Empathy:** Validate the child's feelings by acknowledging their frustration. Use statements like 'I know this is hard, but I am here to help.'
- **Break Tasks into Manageable Steps:** Simplify homework into smaller parts, allowing the child to experience manageable successes.

Scenario 4: Post-Holiday Return

Situation: A child who was managing school attendance before the summer holidays now refuses to return in September.

- **Understand the Transition Impact:** Multiple changes, including new teacher, new classroom, and new expectations, have compounded uncertainty and activated threat responses.
- **Address Anticipatory Anxiety:** The long break allowed anxiety to build. Graduated re-exposure before term starts can help.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

- **Rebuild Safety Cues:** Identify what felt safe in the previous year and how this can be recreated or maintained in the new context.

Section 16: Quick-Reference Guide for Parents and Teachers

This quick-reference guide provides practical strategies to help parents and teachers support children by understanding their emotional responses, managing automatic affects, and positively influencing scripts.

1. **Leverage Strengths and Interests:** Engaging a child's strengths and interests triggers positive affects such as joy and excitement, helping to break negative scripts.
2. **Create Safety:** Maintain consistent routines, provide clear instructions, and offer choices to give the child a sense of control and predictability.
3. **Encourage Emotional Expression:** Allow children to express their emotions freely. Use journaling, art, or conversation as emotional outlets.
4. **Avoid Suppressing Emotions:** Model healthy emotional expression and guide the child through calming techniques when needed.
5. **Focus on Effort:** Praise effort and perseverance rather than just outcomes. Highlight what was done well and discuss what can be learned from mistakes.
6. **Build Relationships:** Trusting relationships are crucial. Spend quality time, listen actively, and demonstrate empathy and patience.
7. **Provide Manageable Steps:** Break tasks into smaller, achievable steps to reduce overwhelm. Use visual aids or checklists where helpful.
8. **Use Positive Reinforcement:** Reward small achievements consistently to encourage repetition of positive behaviours.
9. **Validate Feelings:** Acknowledge the child's emotions, even when difficult. Statements like 'I understand why you feel that way' help the child feel seen.
10. **Work WITH:** Involve the child in problem-solving and decision-making. Their voice matters and they are experts in their own experience.

Remember: Every interaction is an opportunity to either reinforce negative scripts or help build positive ones. Small, consistent positive experiences accumulate to create meaningful change.

© Dr Scott I-Patrick DEdPsych CPsychol 2024 | EdPsych Connect

Section 17: Resources and References

Recommended Reading

- Nathanson, D. L. (1992). *Shame and Pride: Affect, Sex, and the Birth of the Self.* W.W. Norton & Company.
- Tomkins, S. S. (2008). *Affect Imagery Consciousness: The Complete Edition.* Springer Publishing.
- Porges, S. W. (2011). *The Polyvagal Theory: Neurophysiological Foundations of Emotions, Attachment, Communication, and Self-Regulation.* W.W. Norton & Company.
- Dana, D. (2018). *The Polyvagal Theory in Therapy: Engaging the Rhythm of Regulation.* W.W. Norton & Company.
- Wachtel, T. (2016). *Defining Restorative.* International Institute for Restorative Practices.

Useful Websites

- National Autistic Society: www.autism.org.uk
- MindEd (Children's Mental Health): www.minded.org.uk
- Not Fine in School: www.notfineinschool.co.uk
- Square Peg: www.teamsquarepeg.org

Academic References

- George, G. (2014). Affect and Emotion in a Restorative School. In V. Kelly & M. Thorsborne (Eds.), The Psychology of Emotions in Restorative Practice (pp. 200–231). Jessica Kingsley.
- Henderson, D. X., & Guy, B. (2017). Social connectedness and its implication on student-teacher relationships and suspension. Preventing School Failure, 61(1), 39–47.
- Ighavongbe-Patrick, S. (2022). What can children and young people tell us about school sanctions and social relationships? (Doctoral thesis, University of Southampton).
- Thambirajah, M. S., Grandison, K. J., & De-Hayes, L. (2008). Understanding School Refusal: A Handbook for Professionals in Education, Health and Social Care. Jessica Kingsley Publishers.

© Dr Scott I-Patrick DEdPsych CPsychol 2024

EdPsych Connect Limited
This document may not be reproduced without permission.